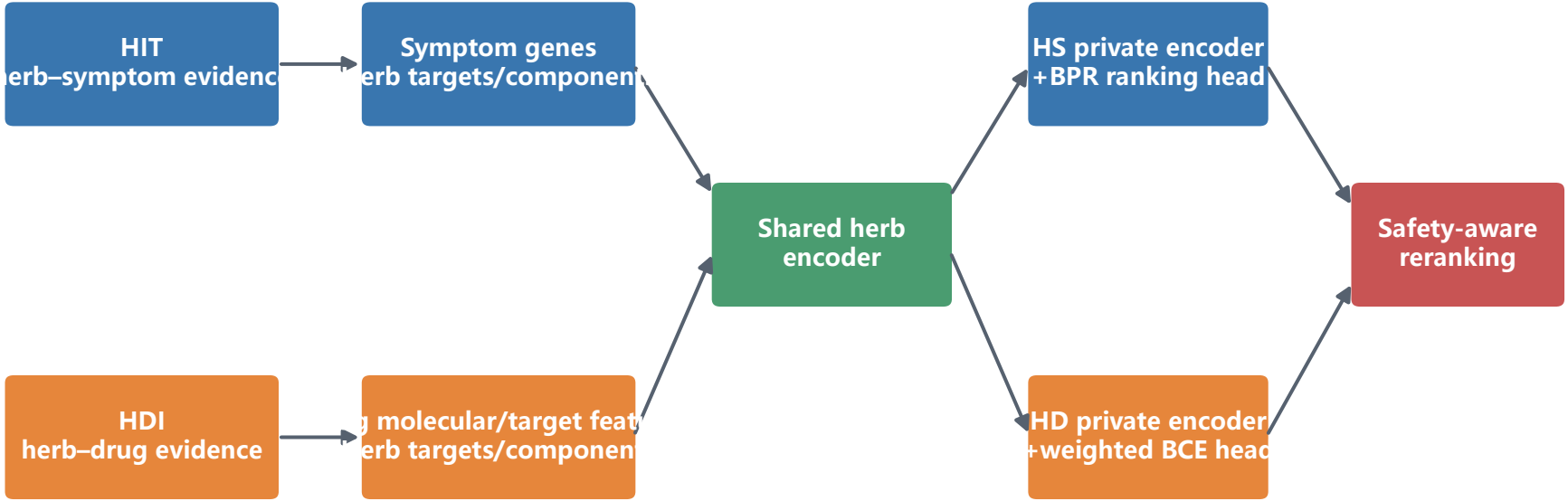

Two binary evidence tasks are combined at decision time; no direct symptom–herb–drug ternary labels are assumed.